Thyrocare

Tests you can trust

**Name** : Charisma Gangavaram(34Y/F)

**Date** : 10 Dec 2024

**Test Asked** : Executive Health Checkup Below 40 Yrs With Utsh

**Report Status:** Complete Report

**9 out of 10 Doctors trust** that Thyrocare **reports are accurate & reliable***

**98% Reports** released within **06 Hours** of sample reaching the lab+

**Samples** Processed in **NABL Accredited** Labs#

**700+** Tests & Profiles

Temperature-Controlled Sample Logistics

Unique Barcode Tracking & Reports with QR Code Verification

Fully Automated Machines Inspected Daily

Abnormal Values Re-Checked Twice

Reports Verified By Expert MD Pathologists Stationed at Every Lab

Accredited by

NABL From 2005#

ISO 9001: 2015 - From 2015

CAP From 2007

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HRBR 2nd Block,
Hennur, Bengaluru-560043

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**NAME** : CHARISMA GANGAVARAM(34Y/F)
**REF. BY** : SELF
**TEST ASKED** : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH

**HOME COLLECTION :**
K210 MOUNIKA ESPANCIA BANJARA LAYOUT
HORAMAVU AGARA BANGALORE 560043
BANASWADI BANGALORE KARNATAKA INDIA

## Report Availability Summary

**Note:** Please refer to the table below for status of your tests.

**10** Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

| TEST DETAILS | REPORT STATUS |
|---|---|
| **EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH** | Ready |
| FASTING BLOOD SUGAR(GLUCOSE) | Ready |
| HEMOGRAM | Ready |
| URIC ACID | Ready |
| TSH - ULTRASENSITIVE | Ready |
| COMPLETE URINE ANALYSIS | Ready |
| HBA PROFILE | Ready |
| LIPID PROFILE | Ready |
| KIDPRO WITHOUT CALCIUM | Ready |
| SERUM SGOT-SGPT RATIO | Ready |
| BILIRUBIN - TOTAL | Ready |

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**HOME COLLECTION :**
K210 MOUNIKA ESPANCIA BANJARA LAYOUT HORAMAVU AGARA BANGALORE 560043 BANASWADI BANGALORE KARNATAKA INDIA 560043-560043

**MOBILE NO** : 9916158018
**DOB** : 09/16/1990

**PAN ID** : HB1-25057

## Summary Report

### Tests outside reference range

| TEST NAME | OBSERVED VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|
| **COMPLETE HEMOGRAM** | | | |
| LYMPHOCYTE | 40.2 | % | 20-40 |
| MEAN CORP.HEMO.CONC(MCHC) | 30.9 | g/dL | 31.5-34.5 |
| MONOCYTES - ABSOLUTE COUNT | 0.18 | X $10^3$ / µL | 0.2 - 1.0 |
| **COMPLETE URINE ANALYSIS** | | | |
| CRYSTALS | CALCIUM OXALATE CRYSTALS | - | Absent |
| **LIPID** | | | |
| HDL CHOLESTEROL - DIRECT | 61 | mg/dL | 40-60 |
| LDL / HDL RATIO | 0.9 | Ratio | 1.5-3.5 |
| TC/ HDL CHOLESTEROL RATIO | 2 | Ratio | 3 - 5 |

**Disclaimer**: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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**DOB** : 09/16/1990
**PAN ID** : HB1-25057

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| **HbA1c - (HPLC)** | | | |
| | H.P.L.C | 4.9 | % |

**Bio. Ref. Interval. :**

| Bio. Ref. Interval.: As per ADA Guidelines | Guidance For Known Diabetics |
|---|---|
| Below 5.7% : Normal | Below 6.5% : Good Control |
| 5.7% - 6.4% : Prediabetic | 6.5% - 7% : Fair Control |
| >=6.5% : Diabetic | 7.0% - 8% : Unsatisfactory Control |
| | >8% : Poor Control |

**Method :** Fully Automated H.P.L.C method

| | | | |
|---|---|---|---|
| AVERAGE BLOOD GLUCOSE (ABG) | CALCULATED | 94 | mg/dL |

**Bio. Ref. Interval. :**

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

**Method :** Derived from HBA1c values

**Please correlate with clinical conditions.**

**Sample Collected on (SCT)** : 10 Dec 2024 10:57
**Sample Received on (SRT)** : 10 Dec 2024 12:10
**Report Released on (RRT)** : 10 Dec 2024 14:22
**Sample Type** : EDTA Whole Blood
**Labcode** : 1012072799/HCL01
**Barcode** : DJ320824

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

**NAME** : CHARISMA GANGAVARAM(34Y/F)
**REF. BY** : SELF
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| TEST NAME | METHODOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| **HEMOGLOBIN** | SLS-Hemoglobin Method | 12.5 | g/dL | 12.0-15.0 |
| Hematocrit (PCV) | CPH Detection | 40.5 | % | 36.0-46.0 |
| Total RBC | HF & EI | 4.59 | X 10^6/µL | 3.8-4.8 |
| Mean Corpuscular Volume (MCV) | Calculated | 88.2 | fL | 83.0-101.0 |
| Mean Corpuscular Hemoglobin (MCH) | Calculated | 27.2 | pq | 27.0-32.0 |
| **Mean Corp.Hemo. Conc (MCHC)** | **Calculated** | **30.9** | **g/dL** | **31.5-34.5** |
| Red Cell Distribution Width (RDW - CV) | Calculated | 13.7 | % | 11.6-14.0 |
| **TOTAL LEUCOCYTE COUNT (WBC)** | HF & FC | 6 | X $10^3$ / µL | 4.0 - 10.0 |
| **DIFFERENTIAL LEUCOCYTE COUNT** | | | | |
| Neutrophils Percentage | Flow Cytometry | 55.1 | % | 40-80 |
| **Lymphocytes Percentage** | **Flow Cytometry** | **40.2** | **%** | **20-40** |
| Monocytes Percentage | Flow Cytometry | 3 | % | 2-10 |
| Eosinophils Percentage | Flow Cytometry | 1.2 | % | 1-6 |
| Basophils Percentage | Flow Cytometry | 0.3 | % | 0-2 |
| Immature Granulocyte Percentage (IG%) | Flow Cytometry | 0.2 | % | 0.0-0.4 |
| Nucleated Red Blood Cells % | Flow Cytometry | 0.1 | % | 0.0-5.0 |
| **ABSOLUTE LEUCOCYTE COUNT** | | | | |
| Neutrophils - Absolute Count | Calculated | 3.31 | X $10^3$ / µL | 2.0-7.0 |
| Lymphocytes - Absolute Count | Calculated | 2.41 | X $10^3$ / µL | 1.0-3.0 |
| **Monocytes - Absolute Count** | **Calculated** | **0.18** | **X $10^3$ / µL** | **0.2 - 1.0** |
| Basophils - Absolute Count | Calculated | 0.02 | X $10^3$ / µL | 0.02 - 0.1 |
| Eosinophils - Absolute Count | Calculated | 0.07 | X $10^3$ / µL | 0.02 - 0.5 |
| Immature Granulocytes (IG) | Calculated | 0.01 | X $10^3$ / µL | 0.0-0.3 |
| Nucleated Red Blood Cells | Calculated | 0.006 | X $10^3$ / µL | 0.0-0.5 |
| **PLATELET COUNT** | HF & EI | 289 | X $10^3$ / µL | 150-410 |

**Clinical history is asked for all the relevant abnormalities detected and in absence / failure of receiving of clinical history, results are rechecked twice and released. Advised clinical correlation.**
**Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)**
**(Reference :** *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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**REF. BY** : SELF
**TEST ASKED** : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH

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K210 MOUNIKA ESPANCIA BANJARA LAYOUT
HORAMAVU AGARA BANGALORE 560043
BANASWADI BANGALORE KARNATAKA INDIA
560043-560043

**MOBILE NO** : 9916158018 **DOB** : 09/16/1990 **PAN ID** : HB1-25057

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| FASTING BLOOD SUGAR(GLUCOSE) | PHOTOMETRY | 83.97 | mg/dL |

**Bio. Ref. Interval. :-**

| As per ADA Guideline: Fasting Plasma Glucose (FPG) | |
|---|---|
| **Normal** | 70 to 100 mg/dl |
| **Prediabetes** | 100 mg/dl to 125 mg/dl |
| **Diabetes** | 126 mg/dl or higher |

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

**Please correlate with clinical conditions.**
**Method:-** GOD-PAP METHOD

**Sample Collected on (SCT)** : 10 Dec 2024 10:57
**Sample Received on (SRT)** : 10 Dec 2024 12:12
**Report Released on (RRT)** : 10 Dec 2024 14:04
**Sample Type** : FLUORIDE PLASMA
**Labcode** : 1012073017/HCL01
**Barcode** : DI080738

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560043-560043

**MOBILE NO** : 9916158018
**DOB** : 09/16/1990

**PAN ID** : HB1-25057

| TEST NAME | METHODOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| **Complete Urinogram** | | | | |
| **Physical Examination** | | | | |
| VOLUME | Visual Determination | 3 | mL | - |
| COLOUR | Visual Determination | Yellow | - | Pale Yellow |
| APPEARANCE | Visual Determination | CLEAR | - | Clear |
| SPECIFIC GRAVITY | pKa change | > 1.030 | - | 1.003-1.030 |
| PH | pH indicator | 5.5 | - | 5-8 |
| **Chemical Examination** | | | | |
| URINARY PROTEIN | PEI | ABSENT | mg/dL | Absent |
| URINARY GLUCOSE | GOD-POD | ABSENT | mg/dL | Absent |
| URINE KETONE | Nitroprusside | ABSENT | mg/dL | Absent |
| URINARY BILIRUBIN | Diazo coupling | ABSENT | mg/dL | Absent |
| UROBILINOGEN | Diazo coupling | Normal | mg/dL | <=0.2 |
| BILE SALT | Hays sulphur | ABSENT | - | Absent |
| BILE PIGMENT | Ehrlich reaction | ABSENT | - | Absent |
| URINE BLOOD | Peroxidase reaction | ABSENT | - | Absent |
| NITRITE | Diazo coupling | ABSENT | - | Absent |
| LEUCOCYTE ESTERASE | Esterase reaction | ABSENT | - | Absent |
| **Microscopic Examination** | | | | |
| MUCUS | Microscopy | ABSENT | - | Absent |
| RED BLOOD CELLS | Microscopy | ABSENT | cells/HPF | 0-5 |
| URINARY LEUCOCYTES (PUS CELLS) | Microscopy | ABSENT | cells/HPF | 0-5 |
| EPITHELIAL CELLS | Microscopy | 5 | cells/HPF | 0-5 |
| CASTS | Microscopy | ABSENT | - | Absent |
| **CRYSTALS** | **Microscopy** | **CALCIUM OXALATE CRYSTALS** | **-** | **Absent** |
| BACTERIA | Microscopy | ABSENT | - | Absent |
| YEAST | Microscopy | ABSENT | - | Absent |
| PARASITE | Microscopy | ABSENT | - | Absent |

**(Reference** : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

**Sample Collected on (SCT)** : 10 Dec 2024 10:57
**Sample Received on (SRT)** : 10 Dec 2024 16:28
**Report Released on (RRT)** : 10 Dec 2024 18:20
**Sample Type** : URINE
**Labcode** : 1012091231/HCL01
**Barcode** : DA447932

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**NAME** : CHARISMA GANGAVARAM(34Y/F)
**REF. BY** : SELF
**TEST ASKED** : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH

**HOME COLLECTION :**
K210 MOUNIKA ESPANCIA BANJARA LAYOUT HORAMAVU AGARA BANGALORE 560043 BANASWADI BANGALORE KARNATAKA INDIA 560043-560043

**MOBILE NO** : 9916158018
**DOB** : 09/16/1990

**PAN ID** : HB1-25057

| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| TOTAL CHOLESTEROL | PHOTOMETRY | 122 | mg/dL | < 200 |
| **HDL CHOLESTEROL - DIRECT** | **PHOTOMETRY** | **61** | **mg/dL** | **40-60** |
| LDL CHOLESTEROL - DIRECT | PHOTOMETRY | 53 | mg/dL | < 100 |
| TRIGLYCERIDES | PHOTOMETRY | 61 | mg/dL | < 150 |
| **TC/ HDL CHOLESTEROL RATIO** | **CALCULATED** | **2** | **Ratio** | **3 - 5** |
| TRIG / HDL RATIO | CALCULATED | 1 | Ratio | < 3.12 |
| **LDL / HDL RATIO** | **CALCULATED** | **0.9** | **Ratio** | **1.5-3.5** |
| HDL / LDL RATIO | CALCULATED | 1.15 | Ratio | > 0.40 |
| VLDL CHOLESTEROL | CALCULATED | 12.16 | mg/dL | 5 - 40 |

**Please correlate with clinical conditions.**

**Method :**

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

| TOTAL CHOLESTEROL | (mg/dl) | HDL | (mg/dl) | LDL | (mg/dl) | TRIGLYCERIDES | (mg/dl) |
|---|---|---|---|---|---|---|---|
| DESIRABLE | <200 | LOW | <40 | OPTIMAL | <100 | NORMAL | <150 |
| BORDERLINE HIGH | 200-239 | HIGH | >60 | NEAR OPTIMAL | 100-129 | BORDERLINE HIGH | 150-199 |
| HIGH | >240 | | | BORDERLINE HIGH | 130-159 | HIGH | 200-499 |
| | | | | HIGH | 160-189 | VERY HIGH | >500 |
| | | | | VERY HIGH | >190 | | |

**Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.**

**Sample Collected on (SCT)** : 10 Dec 2024 10:57
**Sample Received on (SRT)** : 10 Dec 2024 12:15
**Report Released on (RRT)** : 10 Dec 2024 15:51
**Sample Type** : SERUM
**Labcode** : 1012073479/HCL01
**Barcode** : DI568717

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**MOBILE NO** : 9916158018
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| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| BLOOD UREA NITROGEN (BUN) | PHOTOMETRY | 10.7 | mg/dL | 7.94 - 20.07 |
| CREATININE - SERUM | PHOTOMETRY | 0.55 | mg/dL | 0.55-1.02 |
| BUN / SR.CREATININE RATIO | CALCULATED | 19.45 | Ratio | 9:1-23:1 |
| UREA (CALCULATED) | CALCULATED | 22.9 | mg/dL | Adult : 17-43 |
| UREA / SR.CREATININE RATIO | CALCULATED | 41.63 | Ratio | < 52 |
| URIC ACID | PHOTOMETRY | 3.67 | mg/dL | 3.2 - 6.1 |

**Please correlate with clinical conditions.**

**Method :**

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
URIC - Uricase / Peroxidase Method

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|---|---|---|---|---|
| BILIRUBIN - TOTAL | PHOTOMETRY | 0.63 | mg/dL | 0.3-1.2 |
| ASPARTATE AMINOTRANSFERASE (SGOT ) | PHOTOMETRY | 14.1 | U/L | < 31 |
| ALANINE TRANSAMINASE (SGPT) | PHOTOMETRY | 10.1 | U/L | < 34 |
| SGOT / SGPT RATIO | CALCULATED | 1.4 | Ratio | < 2 |

**Please correlate with clinical conditions.**

**Method :**

BILT - Vanadate Oxidation
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.

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| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| TSH - ULTRASENSITIVE | E.C.L.I.A | 0.947 | µIU/mL | 0.54-5.30 |

**Comments :**

**The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.**

**Method :**

USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Pregnancy reference ranges for TSH/USTSH :

| Trimester | T3 (ng/dl) | T4 (µg/dl) | TSH/USTSH (µIU/ml) |
|---|---|---|---|
| 1st | 83.9-196.6 | 4.4-11.5 | 0.1-2.5 |
| 2nd | 86.1-217.4 | 4.9-12.2 | 0.2-3.0 |
| 3rd | 79.9-186 | 5.1-13.2 | 0.3-3.5 |

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

**Disclaimer :**Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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**Report Released on (RRT)** : 10 Dec 2024 15:51
**Sample Type** : SERUM
**Labcode** : 1012073479/HCL01
**Barcode** : DI568717

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

**NAME** : CHARISMA GANGAVARAM(34Y/F)
**REF. BY** : SELF
**TEST ASKED** : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH

**HOME COLLECTION :**
K210 MOUNIKA ESPANCIA BANJARA LAYOUT
HORAMAVU AGARA BANGALORE 560043
BANASWADI BANGALORE KARNATAKA INDIA
560043-560043

**MOBILE NO** : 9916158018 **DOB** : 09/16/1990 **PAN ID** : HB1-25057

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| EST. GLOMERULAR FILTRATION RATE (eGFR) | CALCULATED | 122 | mL/min/1.73 m2 |

**Bio. Ref. Interval. :-**

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

**Please correlate with clinical conditions.**
**Method:-** CKD-EPI Creatinine Equation

**~~ End of report ~~**

**Sample Collected on (SCT)** : 10 Dec 2024 10:57
**Sample Received on (SRT)** : 10 Dec 2024 12:15
**Report Released on (RRT)** : 10 Dec 2024 15:51
**Sample Type** : SERUM
**Labcode** : 1012073479/HCL01
**Barcode** : DI568717

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

## CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.

## EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.